

Living Well with IBS-C

A practical, non-medication guide to irritable bowel syndrome with constipation.



IBS-C means recurring belly pain linked with constipation or hard stools. It is real, it is common, and it responds better to several small, steady changes than to one dramatic fix. Your gut is a bit like a smoke alarm set too sensitive. Nothing is burning, but it keeps going off. Most of what follows is about turning that sensitivity down.

Give any change at least 2 to 4 weeks before judging it. Change one thing at a time so you know what actually helped.

Get checked first if you notice any of these

- Blood in the stool, or black stools
- Weight loss you cannot explain, fever, repeated vomiting, or anaemia
- Symptoms that wake you from sleep
- A new change in bowel habit, especially if you are older
- A family history of colon cancer, inflammatory bowel disease, or celiac disease
- Severe or worsening pain, marked swelling of the belly, or being unable to pass stool or gas
- Symptoms that began after you started a new medicine

IBS is diagnosed by a doctor from your symptom pattern after other causes are considered. Do not label yourself.

1 Build a rhythm

- Eat at regular times. Skip the long gaps and the huge meals.
- Eat slowly and chew well.
- Keep sleep and wake times steady.
- Keep a symptom diary for 2 to 4 weeks: meals, stool form, pain, bloating, stress, sleep.

Your gut moves most after meals. Use that window for your toilet visit, ideally after breakfast, and never in a rush.

2 Fluids

Drink regularly through the day. Roughly 1.5 to 2 litres suits most adults, more in hot weather. If you have a heart, kidney or liver condition, check with your doctor first.

Water alone will not fix IBS-C if your fibre intake is low. The two work as a pair.

3 Move most days

Aim for 20 to 30 minutes of brisk walking, cycling, swimming, yoga or strength work on most days. Build up slowly. Sudden intense workouts can flare the belly, so ease in.

4 Fibre: choose the right kind

Soluble fibre is the usual first choice in IBS-C. It forms a soft gel in the gut and helps stool pass comfortably. Good sources are ispaghula (isabgol) husk, oats, barley, linseed, chia, and well-tolerated fruits and vegetables.

- Start with a small amount and raise it every few days.
- Take it with a full glass of water.
- Give it 1 to 2 weeks before deciding if it works.
- Go easy on wheat bran and other coarse fibre. It often worsens bloating and pain.

More fibre is not always better. If bloating or pain keeps rising, stop escalating and get it reviewed. Some people need a different approach altogether.

5 Food

No single food causes IBS in everyone. Start simple: smaller meals, less deep-fried and very fatty food, fewer fizzy drinks, and moderate tea, coffee and alcohol if they set you off. Avoid detoxes, fasting and crash diets.

The low-FODMAP diet helps some people with pain and bloating, though the evidence is weaker in IBS-C than in the diarrhoea type. It is a short experiment with three phases, never a lifelong diet.

- **Restrict** for about 4 to 6 weeks.
- **Reintroduce** food groups one at a time and watch your response.
- **Personalise** to the least restrictive diet that keeps you comfortable.

Do this with a dietitian who knows gut disorders. Skip it, or get supervision, if you have ever had an eating disorder, are losing weight, are underweight, or are pregnant.

A gluten-free diet is not routine for IBS. If celiac disease is a possibility, get tested before cutting gluten, since stopping it can hide the result.

6 Toilet habits that help

- Go when the urge comes. Repeatedly postponing trains the gut to stop signalling.
- Raise your feet on a small stool so your knees sit above your hips.
- Relax the belly. Avoid long, forceful straining.
- Judge progress by stool form, not daily frequency. The goal is soft, formed stool that passes comfortably. The Bristol Stool Chart is a handy reference.

If stool is soft but you still struggle to empty, or it feels blocked, tell your doctor. Pelvic floor problems are common and treatable with biofeedback.

7 Calm the gut-brain loop

IBS is not "all in your head". Gut and brain talk constantly through nerves, hormones and immune signals. Stress, poor sleep and anxiety turn the volume up on gut sensations, and gut symptoms feed the anxiety back. Both ends can be worked on.

- 5 to 10 minutes of slow diaphragmatic breathing daily
 - Mindfulness or guided relaxation
 - Gentle yoga
 - Consistent sleep, and a steady, moderate caffeine intake
 - Gut-directed CBT or gut-directed hypnotherapy if symptoms stay troublesome
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8 Supplements and quick fixes

Probiotic evidence is mixed and depends on the strain. If you want to try one, use one product at a time, at the labelled dose, for up to about 12 weeks. Stop if nothing improves. A pricier multi-strain product is not automatically better.

Avoid colon cleanses, unregulated herbal mixes and repeated laxative or enema use without medical advice. Quality and contamination are real concerns. If lifestyle changes are not enough, speak to your doctor about treatment options suited to you.

9 Your step-by-step plan

- 1 Confirm the basics.** Rule out red flags, review your medicines, and start the symptom diary.
 - 2 Lay the foundation.** Regular meals, fluids, movement, soluble fibre, a toilet routine, and cutting only the triggers you have clearly identified.
 - 3 Review at 2 to 6 weeks.** Check stool form, pain, bloating and effort. Keep what helps, drop what does not.
 - 4 Go targeted.** Dietitian-led low-FODMAP trial, gut-directed therapy, pelvic floor assessment, or medical treatment under supervision.
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Disclaimer: This guide is for general education only. It is not medical advice, diagnosis or treatment, and it does not create a doctor-patient relationship. Every person's gut is different. Please consult a qualified healthcare professional about your own symptoms before making changes to your diet, activity or treatment, and seek prompt care if you notice any warning signs listed above.